

NCLEX PRESSURE ULCERS (BEDSORES) OVERVIEW

1. WHAT IS A PRESSURE ULCER?

- A pressure ulcer (bedsore, decubitus ulcer) is skin damage caused by prolonged pressure on the skin.
- This pressure **reduces blood flow**, leading to **skin breakdown and open wounds**.
- Common in patients who are bedridden, immobile, or have poor circulation.

2. CAUSES & RISK FACTORS FOR PRESSURE ULCERS

- ♦ **Prolonged Pressure** – Lying or sitting in one position for too long.
- ♦ **Friction** – Rubbing against a surface (e.g., when moving in bed).
- ♦ **Shearing** – Skin slides in one direction while the body moves in another (e.g., sliding down in bed).
- ♦ **Moisture** – Sweat, urine, or stool **weakens skin** (common in incontinent patients).
- ♦ **Poor Nutrition & Hydration** – Lack of protein, vitamins, and water weakens skin.
- ♦ **Medical Conditions** – Diabetes, poor circulation, or spinal cord injuries increase risk.

3. COMMON LOCATIONS OF PRESSURE ULCERS

- Bony areas (where skin is thin over bones) are at highest risk.
- Common pressure ulcer sites:
 - ✓ Back of head (Occiput)
 - ✓ Shoulders & shoulder blades

- ✓ Elbows
- ✓ Lower back & tailbone (Sacrum & Coccyx)
- ✓ Hips & buttocks
- ✓ Heels & ankles

4. STAGES OF PRESSURE ULCERS

Stage	Skin Damage	Appearance (What It Looks Like?)	Pain?	Healing Time
Stage 1	Skin red but intact	Red, non-blanchable (does not turn white when pressed)	Yes	Days to weeks
Stage 2	Partial-thickness skin loss	Shallow open wound or blister	Yes	Weeks
Stage 3	Full-thickness skin loss (fat layer exposed)	Deep crater, may see yellow tissue (slough)	May or may not hurt	Months
Stage 4	Full-thickness loss with exposed muscle, tendon, or bone	Deep, black (necrotic) tissue or eschar (thick dead tissue)	May not hurt (nerves destroyed)	Months to years
Unstageable	Covered with slough (yellow/green dead tissue) or eschar (black dead tissue), so depth cannot be determined	Wound needs debridement (removal of dead tissue) to stage it	May or may not hurt	Depends on severity
Deep Tissue Injury (DTI)	Damage under intact skin	Purple/maroon area, looks like a deep bruise	Very painful	Can worsen to open wound

5. PREVENTION OF PRESSURE ULCERS (BEST STRATEGY = PREVENTION!)

- ✓ **Reposition Every 2 Hours** – Helps relieve pressure.
- ✓ **Use Pillows or Foam Pads** – Supports bony areas.
- ✓ **Keep Skin Clean & Dry** – Prevents breakdown from moisture.
- ✓ **Apply Barrier Creams (Zinc Oxide, Petroleum Jelly)** – Protects against urine & stool.
- ✓ **Encourage High-Protein Diet** – Protein helps heal skin.
- ✓ **Use Special Mattresses (Low-Air Loss or Foam Mattresses)** – Reduces pressure.
- ✓ **Assess Skin Daily (Especially on Bony Areas).**

6. TREATMENT OF PRESSURE ULCERS

STAGE 1 & 2 (MILD TO MODERATE)

- ✓ **Keep the area clean & dry.**
- ✓ **Apply moisture-retaining dressings (Hydrocolloid, Transparent Film).**
- ✓ **Encourage movement & repositioning.**

STAGE 3 & 4 (SEVERE, DEEP WOUNDS)

- ✓ **Debridement (removal of dead tissue)** – Can be done surgically or with special dressings.
- ✓ **Use Wound Dressings:**
 - **Hydrocolloid (for moderate drainage).**
 - **Foam Dressings (for heavy drainage).**
 - **Alginate (absorbs excessive drainage).**
 - ✓ **Apply Antibiotic Ointment (if infected).**
 - ✓ **Negative Pressure Wound Therapy (Vacuum-Assisted Closure - Wound VAC)** – Helps heal deep wounds.
 - ✓ **May need Skin Grafting (Surgical Skin Replacement) for Large Ulcers.**

7. NCLEX PRIORITIES FOR PRESSURE ULCERS

- ✓ Turn & Reposition Every 2 Hours (Prevention Is Key!).
- ✓ Keep Skin Dry but Moisturized (Moisture Balance is Important!).
- ✓ Use Special Mattresses & Pillows to Reduce Pressure.
- ✓ Encourage a High-Protein, High-Calorie Diet for Healing.
- ✓ Monitor for Signs of Infection (Fever, Redness, Pus, Worsening Pain).

8. COMPLICATIONS OF PRESSURE ULCERS

⚠ Infection (Cellulitis, Osteomyelitis, Sepsis)

- Redness, swelling, warmth, pus = Infection.
- Fever, chills, confusion = Sepsis (life-threatening infection).
- Bone infection (Osteomyelitis) can develop in Stage 4 ulcers!

⚠ Delayed Healing

- Poor nutrition (low protein, vitamin deficiencies) slows healing.
- Diabetes & poor circulation increase risk of non-healing ulcers.

⚠ Pain & Mobility Issues

- Pain from deep ulcers can make movement difficult.
- Contractures (stiff joints) can develop if patients don't move.

9. NCLEX "MUST-KNOW" PRESSURE ULCER CONCEPTS

- ♦ Stage 1 = Red but skin intact (turn every 2 hours!).
- ♦ Stage 2 = Open blister or shallow wound (moist dressings help heal!).
- ♦ Stage 3 = Deep wound with fat showing (needs special wound care!).
- ♦ Stage 4 = Muscle/Bone exposed (may need surgery!).
- ♦ Unstageable = Needs debridement to assess depth!
- ♦ Deep Tissue Injury = Purple/maroon skin = Skin death underneath!

- ♦ Repositioning every 2 hours = Best way to prevent ulcers!
- ♦ High-protein diet = Speeds healing of wounds!
- ♦ Fever + Worsening Ulcer = Possible Sepsis (Call Doctor!).